Vinorelbine capsules · advanced breast cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Vinorelbine capsules · reviewed support plan

Active medicines: Vinorelbine

Emetogenic risk: Moderate-to-high oral risk

Medicine	Dose / route	When
One 5-HT3 antagonist · choose a single option	Ondansetron 8–16 mg total daily OR granisetron 1–2 mg total daily	Around the weekly dose; the prescriber must state the number of subsequent days NCCP Table 5 oral pathway. Do not combine both agents or assume indefinite daily treatment. Review constipation, QT risk and interacting medicines.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by these NCCP monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.
- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.
- Confirm strong CYP3 A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- Table 5 says continue daily; for once-weekly vinorelbine the prescriber must state the number of days. Do not automatically turn this into indefinite daily treatment or import an intravenous (IV) moderate steroid/NK 1 schedule.
- A separate locally approved as-needed plan; check constipation and QT/interactions. Avoid duplicate 5-HT3 agents. Ondansetron can worsen constipation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00259 v6 · p5 · NCCP antiemetic guidance v6 · oral Table 5

Sources checked 2026-10-25 · SACTCheck v0.84.20 · Regimen-level drafts; prescriber and oncology-pharmacy verification required